Bleeding in late Pregnancy

Aim

At the end of this module the student will be able to

Provide an appropriate nursing management according to Bleeding in late Pregnancy

Intended Learning out comes (ILOs):

By the end of this module the student will be able to achieve the following objectives:

Knowledge and Understanding

1. Define bleeding in late pregnancy.
2. Identify the causes of bleeding in late pregnancy
3. Identify types of bleeding in late pregnancy
4. List signs associated with bleeding in late pregnancy
5. Discuss with the client the plan of action to be taken when any of the danger signs associated with bleeding in late pregnancy appear

Intellectual skills:

6. Interpret laboratory results.
7. Report and record the findings.
8. Interpret the findings and report any deviation from normal

Professional and practical skills :

10. Provide an appropriate nursing management of patient of late bleeding

11. Take and record personal data and a complete history.
12. Reassure and calm women with late bleeding in pregnancy
13. Perform a physical examination and recognize signs of late bleeding.
14. Assess the uterine condition.
15. Monitor vital signs and report any deviation from normal.
16. Assess the amount of blood loss.
17. Administer IV fluids and medications according to doctor's orders

Teaching and Learning Activities:

- Lecture
- Brainstorming
- Group discussion
- Case Study and Discussion

Outline

- Definition
- Causes of bleeding in late pregnancy
- Types of bleeding in late pregnancy.
- Signs and symptoms of bleeding in late pregnancy
- History taking of bleeding in late pregnancy
- Physical assessment of bleeding in late pregnancy
- Laboratory investigation.
- Nursing management of bleeding in late pregnancy

Vaginal Bleeding in Late Pregnancy
(Ante partum Hemorrhage)

Definition

Ante partum hemorrhage : is bleeding from the genital tract between 28th week of pregnancy and onset of labor (third trimester bleeding).

Causes of Ante partum Hemorrhage

- **Placental site hemorrhage**

  1) Placenta praevia

  2) Accidental hemorrhage

- **Extra placental hemorrhage**

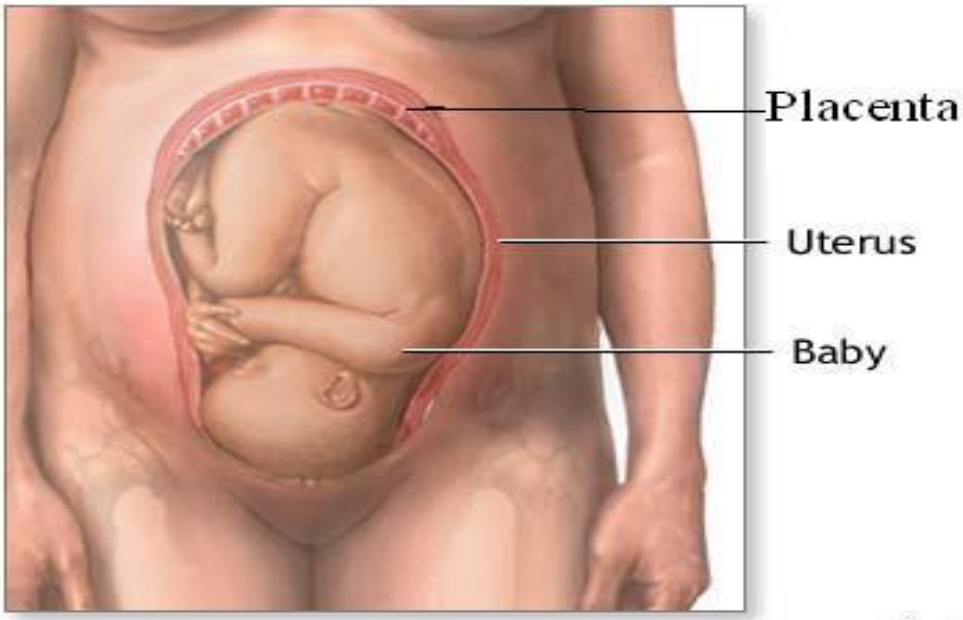
  1) Local genital tract causes

  2) Rupture uterus

Placenta praevia

Definition

Placenta implanted partially or totally in the lower uterine segment so that it is located over or very near the internal os.



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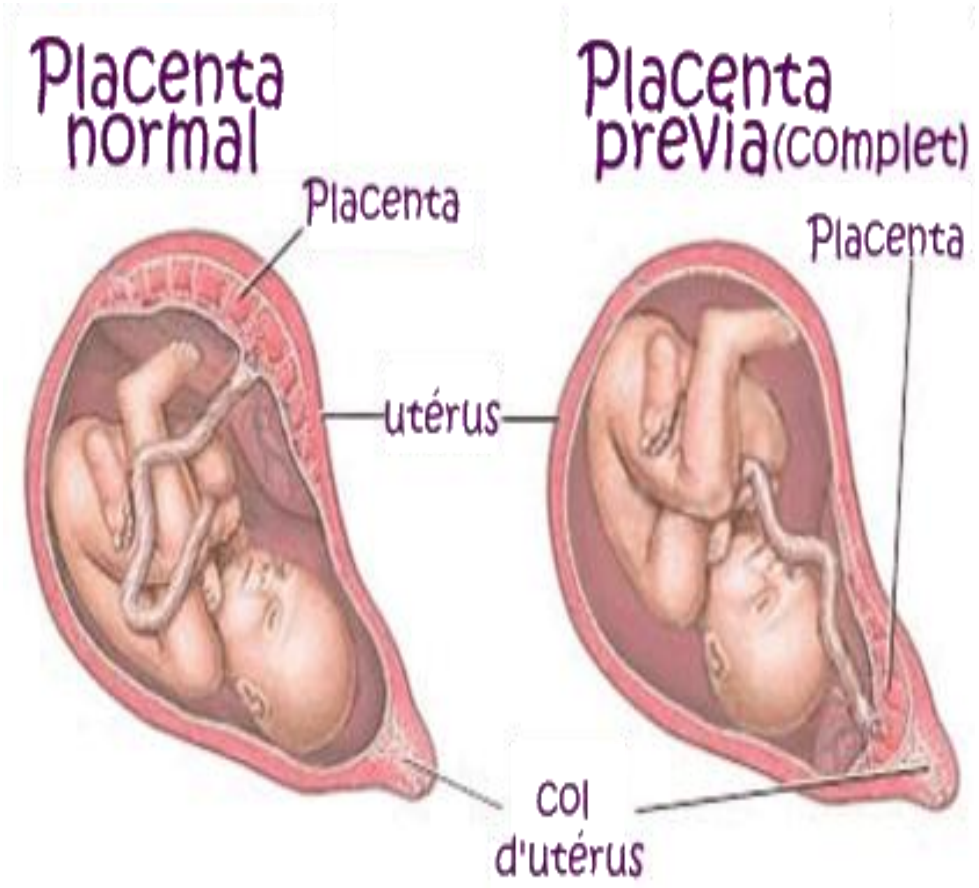
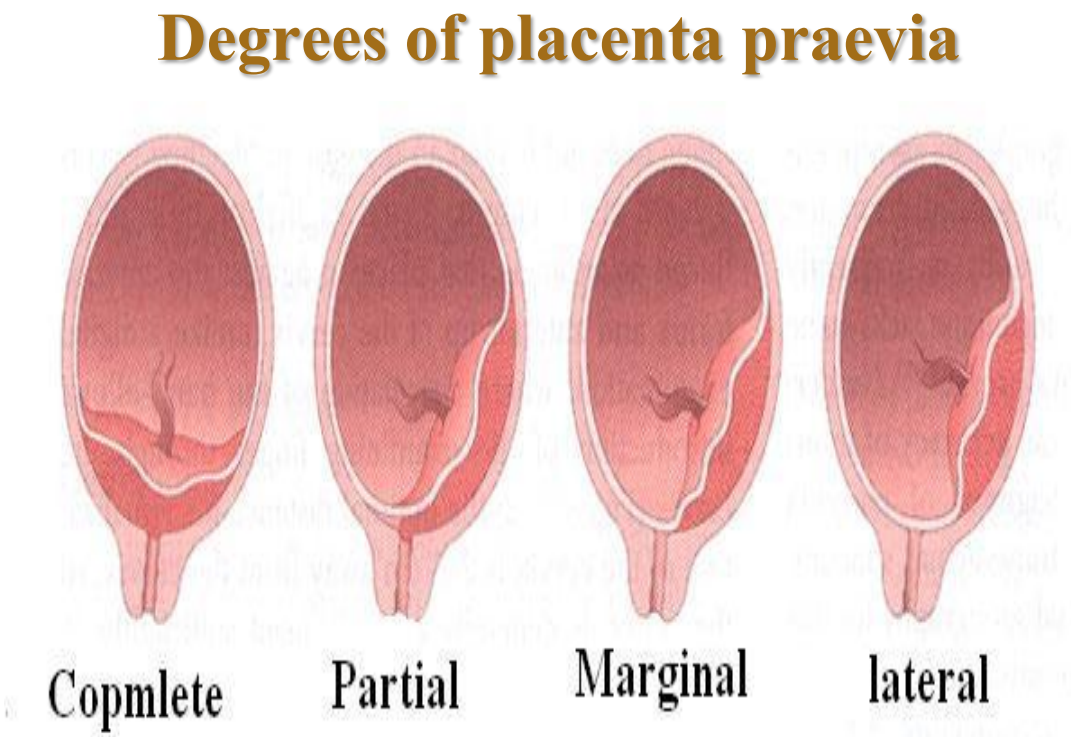


Predisposing factors

- **Large placenta**
  - **Previous uterine scaring as LSCS & deep curettage**
  - **Multipara**
  - **Corpus luteum defect**
  - **Age more than 35 years**
  - **Defective Visualization.**
  - **Twins**
  - **Previous placenta previa**

Degrees of placenta praevia

- **First degree (lateralis): placenta lies in lower uterine segment but away from internal os.**
- **Second degree (marginalis): the placenta can be touched through the cervix but doesn't extended beyond the internal os**
- **Third degree (partial) : incomplete centralized the internal os is partially covered by the placenta.**
- **Fourth degree (total) : the internal os is completely covered by the placenta.**



Diagnosis

1-Symptoms

Bleeding is recurrent, painless and causeless

II-signs

General examination

The general condition of the women depend on the amount of vaginal bleeding .

Abdominal examination

Uterus is normal size, fetal parts are easily palpated, FHS usually audible, malpostion and malpresentation are common,

Vaginal examination

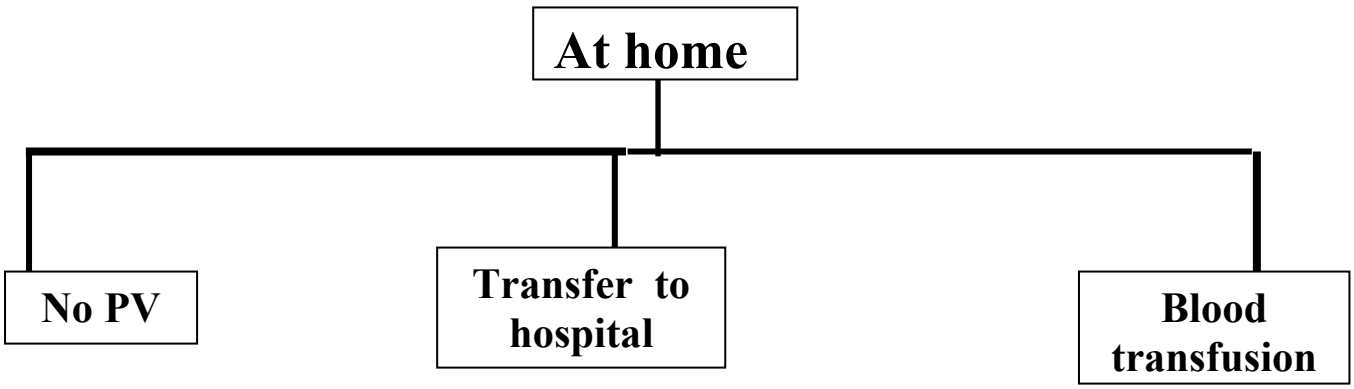
It should only done in the operating room preferably under anesthesia and complete aseptic precautions.

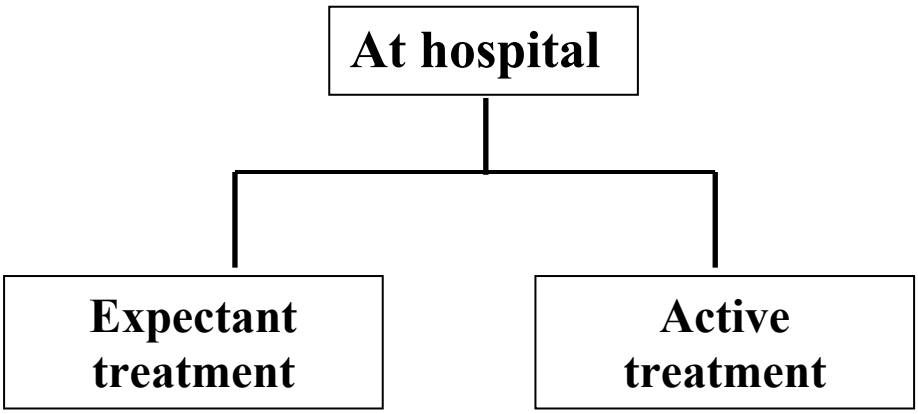
The following should be noted during vaginal examination :-
Amount of bleeding, degree of cervical dilatation, condition of membrane, prolapse of the cord, the presenting part and it's level and estimation of pelvis capacity.

III -Investigation

- **Ultasonography**
- **Radiography (placentography)**
- **Radioactive isotopes**
- **Thermography**

Management





Expectant Management

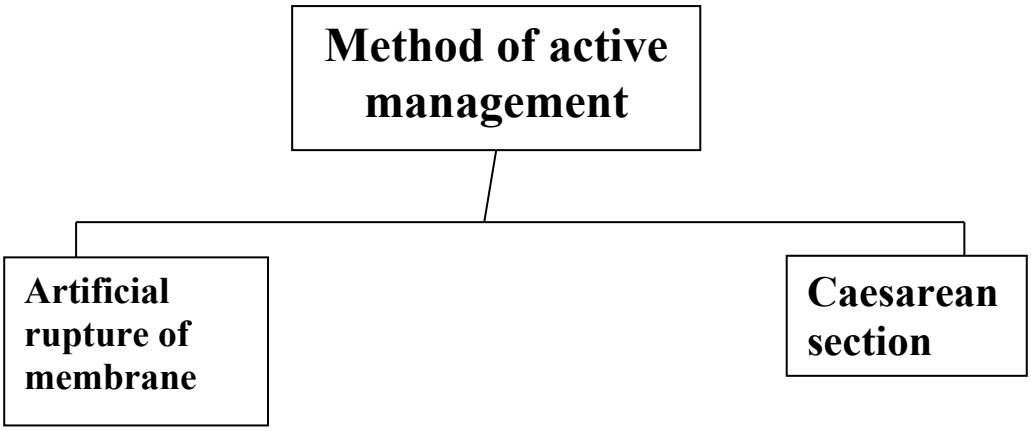
The idea is to delay termination of pregnancy until the fetus is mature enough:-

- General and abdominal examination, blood grouping, blood picture.
- Speculum examination after 48 hours of bleeding to exclude gynecological causes
- Bed rest and correct anemia.
- Ultrasound is done for the following :- to localized the placenta, to determine (viability, maturity, presentation and position of fetus and exclude any abnormality).
- No vaginal examination is done during expectant treatment except :- after 37 weeks of pregnancy, if labor start, if there is an attack of vaginal bleeding.

Active Management

Indication :-

- Patient 1st seen after 37 weeks
- Patient is in labor
- Sever or persistent bleeding



Accidental Hemorrhage (Placenta Abruption)

Definition

Premature separation of normally implanted placenta from endometrium

Risk factors

- Increased Maternal age and parity
- Preeclampsia
- Chronic hypertension
- PROM
- Smoking
- Prior abruption
- Multiple pregnancy
- Trauma
- Anaemia
- Polyhydramnios
- Previous Caesarean Section
- Most have no known cause – presumed late implantation

Signs and symptoms

- vaginal bleeding
- severe abdominal pain, which is different from uterine contractions
- uterus is tender and contracted tightly
- fetal distress, detected by fetal monitoring
- constant contractions, called uterine tetany
- back pain
- uterine tenderness
- maternal shock, with low blood pressure and inadequate blood flow to vital organs

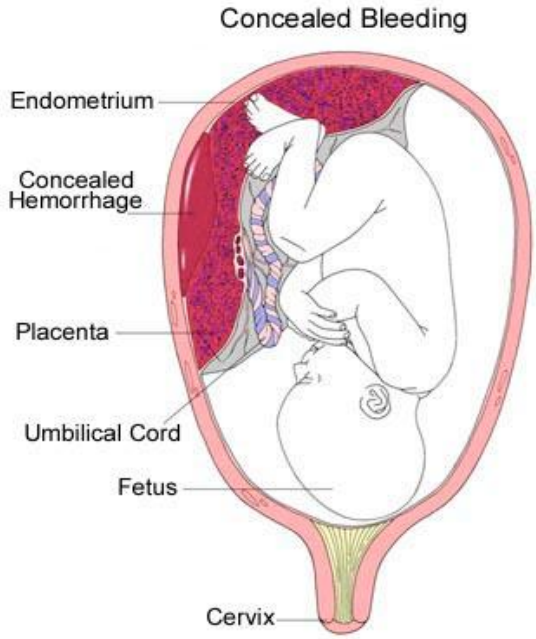
Types of placenta abruption

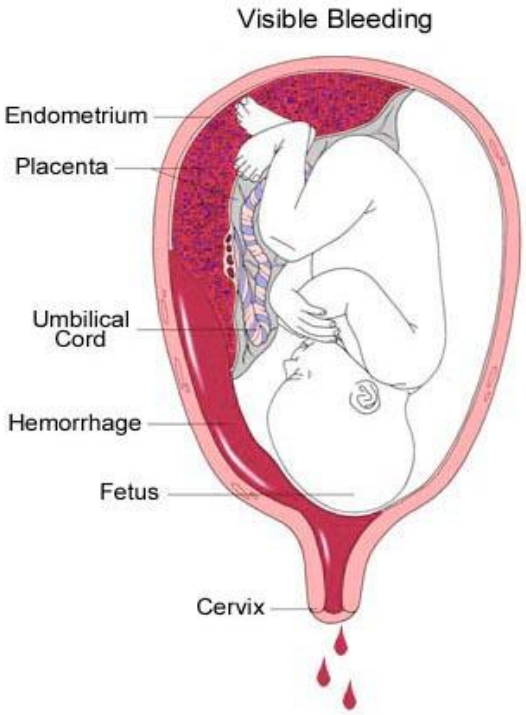
- **Grad 1:-not recognized , before delivery, diagnosed by retro-placental clot.**

- **Grad 2:- intermittent, signs of abruptio placenta present but fetus is alive**

- **Grad 3:- all signs of abruptio placenta present and fetus is dead.**

Types according to vaginal bleeding





<u>Types according to severity</u>

- **Grad 1:-not recognized , before delivery, diagnosed by retro-placental clot.**

- **Grad 2:- intermittent, signs of abruptio placenta present but fetus is alive**
- **Grad 3:- all signs of abruptio placenta present and fetus is dead.**

<u>Management of accidental hemorrhage</u>

Treatment at home

- No vaginal examination
- History taking
- Transfer to hospital

Treatment at hospital

- Initial infusion of RBCs
- Full investigation as pp
- Treatment depend on severity of case, fetal condition, type of bleeding

Treatment revealed type

Not in labor

- Sever Hge CS
- Mild hemorrhage – conservative TT
- fetus-A.R.O.M

In labor

A.R.O.M

Vaginal delivery

Sever bleeding C s

Treatment concealed type

Fetus living

C . S

Fetus dead

Blood transfusion

Induction by oxytocin & A.R.O.M

If failed - C.S

If succeed- vaginal delivery

PLACENTA PREVIA VERSUS ABRUPTIO PLACENTAE		
Assessment	**Placenta Previa**	**Abruptio Placentae**
Pain	Painless	Uterine tenderness; severe abdominal pain and possibly aching or dull pain in the lower back
Bleeding	Bright red	May be concealed; if noted, it is often dark red
Uterus	No unusual contractions or irritability	"Board-like" abdomen; uterine irritability with poor resting tone
Risk for postpartum hemorrhage	High risk; due to low placement of the placenta, there is limited uterine contraction	High risk due to poor contractility of the uterus following an abruption

Summary

- Late pregnancy bleeding may herald diagnoses with significant morbidity/ mortality
- Determining diagnosis important, as treatment dependent on cause
- Avoid vaginal exam when placental location not known

Reference

Cunningham FG, McDonalds PC, Gant NF, et al.Williams Obstetrics, 22th edn. Stamford, Connecticut: Appleton & Lange, 2020

Duff P, Edwards RK, Davis JD, et al. Obstetrics & Gynaecology: Just the Facts. New York: McGraw-Hill, 2019Figure 3 Examples of Vaginal Bleeding in late pregnancy.

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